



Medreach

RIGHT HOSPITAL · RIGHT NOW

The right hospital, right now, **confirmed before you leave home**

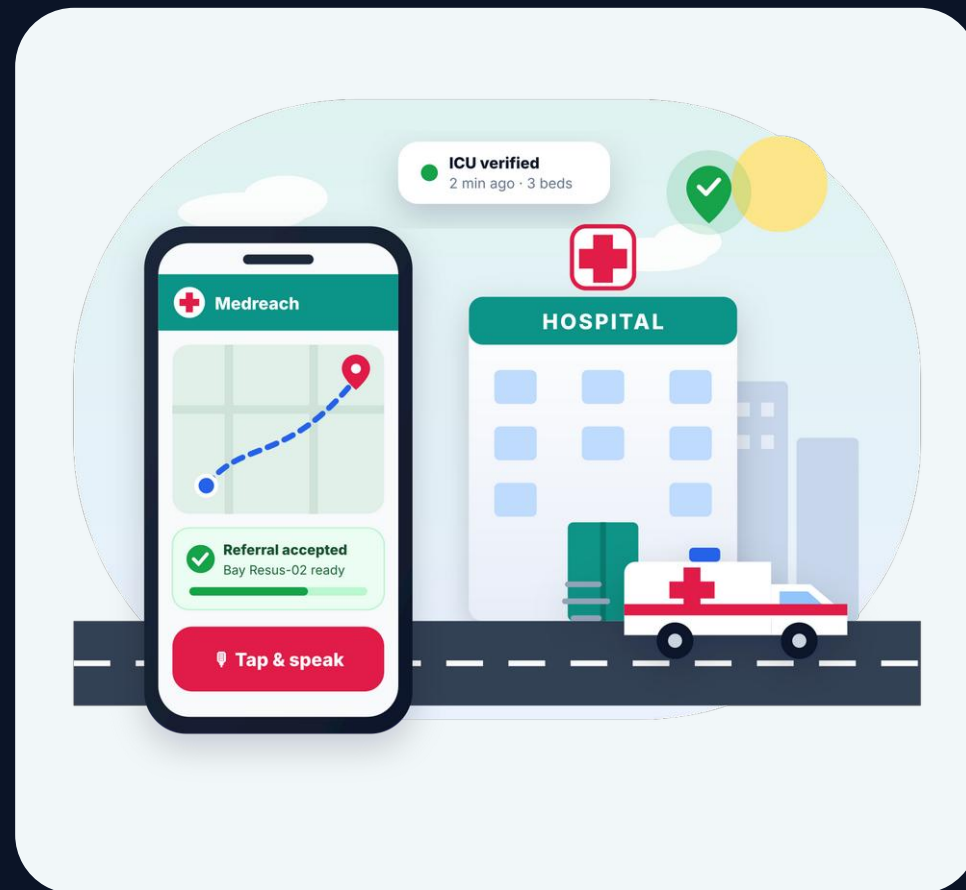
AI-powered emergency coordination for Madhya Pradesh: understand the emergency, find a hospital that can treat it now, get it to accept, and guide the patient there.

MPOnline Hackathon 2026

Challenge 5 · Healthcare

Team Leader: Prakhar Porwal

Live demo: medreach-wtne.onrender.com · Code: github.com/prakharporwal518-collab/Medreach



Introduction: what Medreach is

Medreach is an AI-powered emergency coordination platform for Madhya Pradesh. It understands an emergency described in Hindi, English or Hinglish, finds a hospital that can treat it right now using staff-verified data, gets that hospital to accept the patient, and guides the patient there with live tracking.

Challenge 5 · AI Innovation for Public Services & Citizen-Centric Governance · Domain: Healthcare



Citizens & families

Voice-first emergency help, no login needed, “I’m alone” mode



Hospitals

Verified status board, referrals with an AI pre-arrival note



Government of MP

Live, audited view of emergency capacity across hospitals

7 / 7

suggested technologies used

2

private portals: citizen & hospital

16

Bhopal-region hospitals in the demo

51

automated tests passing



Live and deployed: medreach-wtne.onrender.com

Scenario: chest pain at 11 pm in Bhopal

A 62-year-old man has chest pain and is sweating. Here is what happens today, and what happens with Medreach.



Today

- 1 Family drives to the nearest hospital
- 2 No cardiologist on duty, ICU full
- 3 Referred onward: second trip across the city
- 4 ER unprepared; golden hour lost



With Medreach

- 1 Son speaks the emergency in Hindi
- 2 AI: may indicate heart attack, needs cardiologist + ICU
- 3 Bansal Hospital accepts, bay Resus-02 ready
- 4 Ambulance tracked live; ER read the note in advance

Result: one journey instead of two, and the hospital is ready when the patient arrives. (Illustrative scenario; time saved will be measured in the pilot.)

Overview: why patients lose the golden hour



1,68,491

road-accident deaths in India, 2022

Source: MoRTH, "Road Accidents in India 2022". Heart attacks, strokes, burns and obstetric emergencies add many more.



Nearest ≠ capable

Maps don't know if the specialist is on duty, the ICU is free or the CT works now.



No trusted live status

Availability is unpublished or outdated, with no owner and no time stamp.



No acceptance first

Families arrive unannounced; refusals happen at the gate.



Panic & language

Hard to type forms in panic; many prefer to speak in Hindi.



People living alone

Cannot call hospitals, arrange transport and inform family at once.



Lost information

Allergies and ABHA / Ayushman details don't reach the ER doctor.

Who is affected



Patients & families

Wasted trips, refusals, delayed care



Rural citizens & ASHA workers

Long distances make a wrong choice costly



Hospital ER teams

Unannounced critical arrivals, crowding



Health department

No live picture of emergency capacity

Solution: one flow from emergency to confirmed bed

An emergency coordination platform, not a diagnosis app. Every step of our flowchart is implemented and live.



Hindi · English · Hinglish

Voice-first, built for panic

Only capable hospitals

Deterministic, explainable matching

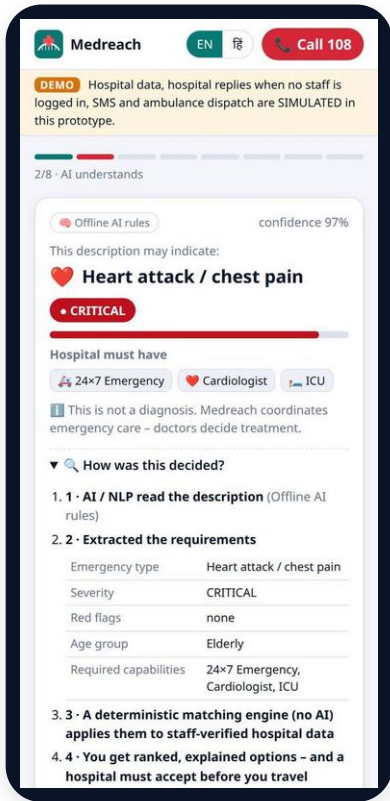
Accepted before travel

No refusals at the gate

60 s auto-fallback

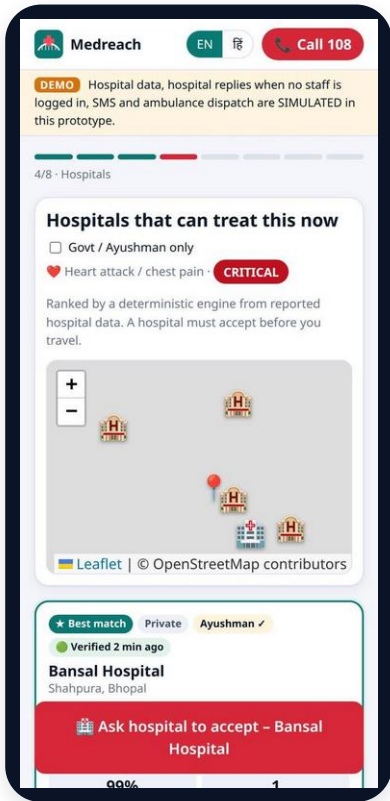
No answer → next hospital asked

Solution in action: citizen and hospital



AI understands

May indicate heart attack · CRITICAL



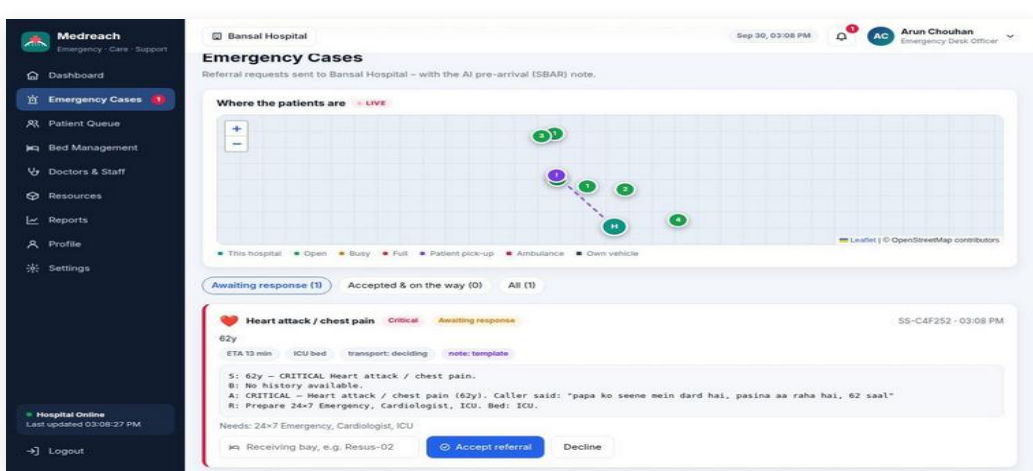
Capable hospitals

Ranked from verified data



Accepted

Named officer, bay Resus-02

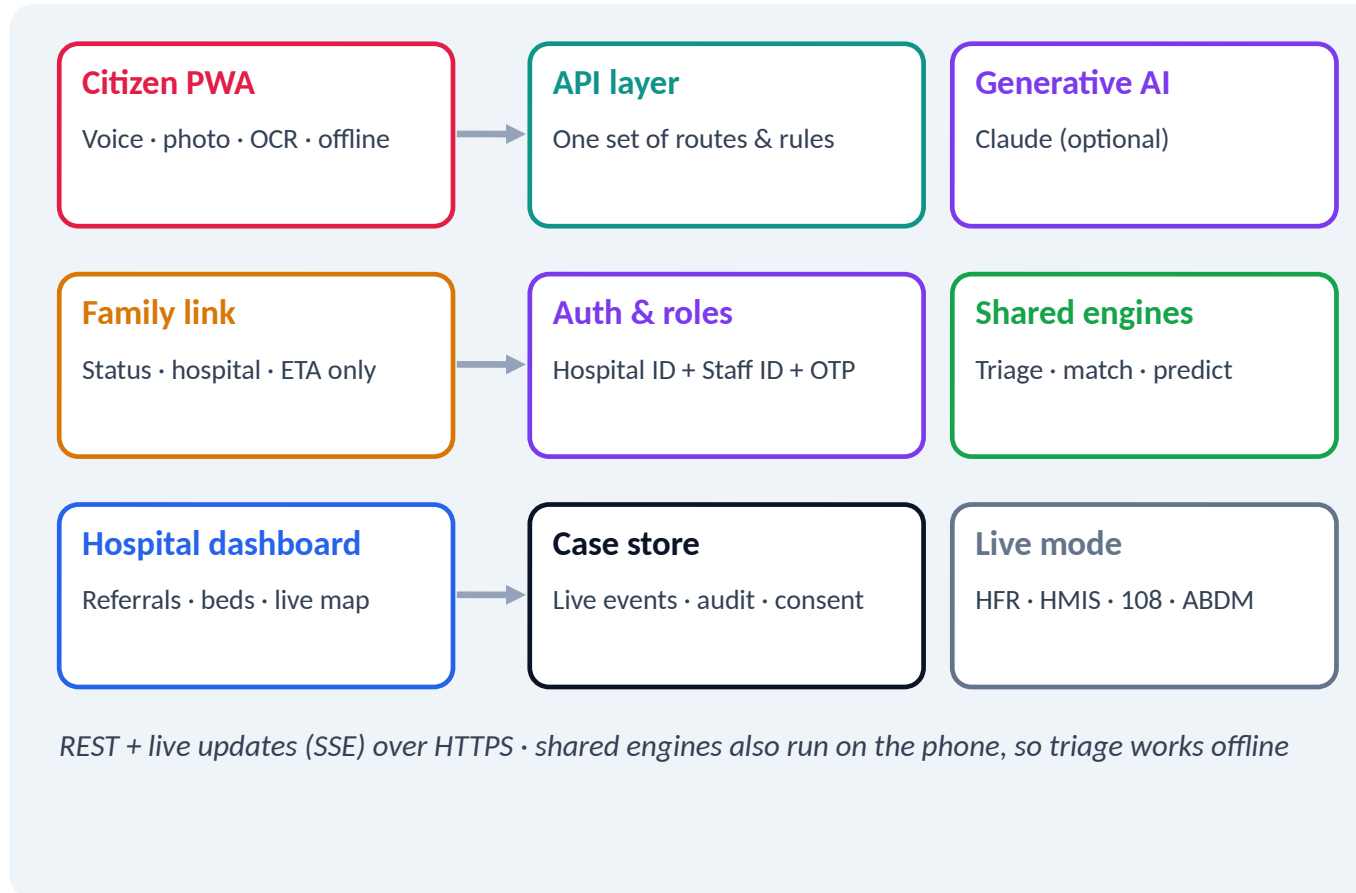


Hospital reviews the AI pre-arrival note and accepts with a receiving bay



Citizen dashboard: live map, ambulance on the way to the accepted hospital

Technical approach: architecture and AI



All 7 technologies in code

-  **Generative AI** requirements + SBAR note
-  **Voice Bots** Hindi / English in and out
-  **Computer Vision** photo of injury → findings
-  **OCR** Ayushman / ABHA card on phone
-  **Predictive Analytics** ETA, bed-on-arrival, forecast
-  **Conversational AI** one follow-up at a time
-  **Mobile App** offline, bilingual PWA

AI understands; rules decide. Severity = max(AI, rules), so AI can never lower urgency. A deterministic, tested engine chooses the hospital.

Implementation: from prototype to statewide



Prototype

Done

Both portals live, AI triage, matching, acceptance, live maps, 51 tests



Pilot • Bhopal

~3 months

Onboard hospitals, nodal officers, ER desk training, clinical review, SMS OTP



Integrate

~3–6 months

Health Facility Registry, HMIS, 108 control room, ABDM, encrypted database



Scale • MP

~6–12 months

District-by-district rollout, state command view, IVR for feature phones

Integrations for live mode



NHA Health Facility Registry



Hospital HMIS feeds



108 control room (CAD)



ABDM / ABHA records



SMS gateway (OTP, alerts)



OpenStreetMap, no API keys

Risk	Mitigation
Hospitals don't update data	Freshness on every figure, stale ranked lower, one-click confirm
Wrong AI reading	AI only raises urgency; deterministic matching; 108 always visible
No hospital answers	60 s timeout → next suitable hospital
Weak network	Offline PWA runs triage & matching on the phone

Innovation: what makes Medreach different

Capability	Map search	Bed portals	Helplines	Medreach
Understands the emergency (voice, Hindi)	No	No	Operator	AI + voice
Matches capability (specialist, equipment)	No	Beds only	Manual	Yes
Shows freshness & who updated data	No	Rarely	No	Every figure
Hospital accepts before travel	No	No	Sometimes	Yes + bay
Automatic fallback to next hospital	No	No	Manual	60 s
AI pre-arrival note for ER doctor	No	No	No	SBAR note
Help for a person living alone	No	No	No	"I'm alone"

Live proof: when a hospital sets its ER to "Diverting", it disappears from every citizen's list within a second.



Three truths kept separate

Reported capacity ≠ referral accepted ≠ confirmed destination.



Explainable score

"Why this hospital?" checklist first; every score part shown.



Privacy by design

Consent, masked IDs, audit log, deletion 24 h after hand-over.



Offline-first

Same engines run on server and phone; works on weak networks.

Impact and benefits



Social

Right care the first time; equal access for rural, elderly and Hindi-speaking citizens



Economic

No repeat trips or wasted visits; better use of ICU beds and specialists



Governance

Live, audited view of emergency capacity for planning beds and ambulances



Educational

Safe first-aid steps, “not a diagnosis” clarity, awareness of Ayushman, ABHA, 108



User

One button, voice, bilingual, clear status and a family tracking link



Health system

Fewer surprise arrivals, less ER crowding, fewer inter-hospital transfers

Benefits by stakeholder



Patient

Treated sooner, bay ready



Family

Live link, no medical data



ER team

Advance notice + AI note



Government

Audited capacity data



108 service

Destination already accepted



ASHA workers

Raise an emergency by voice

Pilot KPIs: time to definitive care · secondary transfers avoided · referral acceptance time · data freshness

Why should Medreach be selected?

Thank you



Real, urgent problem

Solves a life-or-death gap for citizens of Madhya Pradesh: the right hospital, the first time.



Working, deployed product

Both portals live on the web today, not slides or a mock-up.



All 7 technologies

Generative AI, voice, vision, OCR, predictive and conversational AI in a mobile app.



Safe & explainable AI

AI can only raise urgency; a tested engine decides and explains why.



Privacy by design

Consent, masked IDs, audit logs and 24-hour deletion; DPDP Act aligned.



Ready to scale

Clear pilot plan and integration path to HFR, HMIS, 108 and ABDM.

Live demo: medreach-wtne.onrender.com Code: github.com/prakharporwal518-collab/Medreach Hospital login: Bansal Hospital · BANSAL-ED01